8.4 Hyperosmolar Hyperglycemic State

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • Family say she has been drowsy for 2 days **Objective** • Glucose 900 mg/dL • Serum osmolality very high • Hardly any ketones • Mouth dry	**ACTUAL** **Deficient fluid volume** related to severe osmotic diuresis as evidenced by dehydration and high osmolality.	Fluid deficit and glucose corrected gradually over 24–48 hours.	• Give IV fluids as prescribed; this is corrected more slowly than DKA. • Follow sodium and osmolality. • Record urine output every hour. • Watch neurological status.	Better hydrated and more alert: **goal met**.
Subjective • Family say • "She does not recognize us." **Objective** • Confused • GCS 12	**ACTUAL** **Acute confusion** related to hyperosmolality as evidenced by drowsiness and confusion.	Back to her usual orientation as the osmolality corrects.	• GCS every hour. • Keep her safe: bed low, side rails as policy allows. • Reorient her often.	Oriented: **goal met**.
Subjective • *No specific complaint* **Objective** • Dehydrated • Immobile • Elderly	**RISK** **Risk for thrombosis** as evidenced by risk factors: hyperviscosity and immobility.	No VTE.	• Give the prophylactic anticoagulant as ordered. • Get her moving as soon as she can. • Check the calves every day.	No VTE: **goal met**.